ST. JUDE METROPOLITAN HOSPITAL

Department of Inpatient Internal Medicine • Clinical Discharge Summary

Patient: Priya Sharma	**MRN:** MRN-2026-000201	**Admission:** 2026-03-10
Age/Sex: 54 Yrs / Female	**Attending:** Dr. ClinIQ Consultant	**Discharge:** 2026-03-14

ADMISSION DIAGNOSIS & REASON FOR HOSPITALIZATION

Hypertensive urgency with Stage 3 Chronic Kidney Disease exacerbation and symptomatic hyperglycemia (Blood Glucose 284 mg/dL, BP 178/102 mmHg, Serum Creatinine 2.1 mg/dL, eGFR 27 mL/min/1.73m²). Patient presented with headache, bilateral pedal edema, and blurred vision.

HOSPITAL COURSE & INTERVENTIONS

Patient was admitted to the intermediate medical step-down unit. Antihypertensive therapy was initiated with oral amlodipine and uptitration of lisinopril under careful telemetry and daily renal monitoring. Glycemic control was stabilized using subcutaneous basal-bolus insulin sliding scale, transitioning to oral metformin upon renal clearance verification. Serial renal panels demonstrated improvement of serum creatinine from 2.1 mg/dL to 1.55 mg/dL with resolution of acute fluid retention. Bilateral lower extremity ultrasound ruled out deep vein thrombosis.

DISCHARGE VITALS & LAB VALUES

BP: 132/82 mmHg	**HR:** 74 bpm	**Creatinine:** 1.55 mg/dL	**Glucose:** 134 mg/dL

DISCHARGE MEDICATIONS

- Metformin 500mg — 1 tablet PO twice daily with meals
- Lisinopril 10mg — 1 tablet PO once daily each morning
- Amlodipine 5mg — 1 tablet PO once daily each evening
- Atorvastatin 20mg — 1 tablet PO once daily at bedtime

FOLLOW-UP INSTRUCTIONS & RED FLAGS

1. Outpatient follow-up with Dr. ClinIQ Consultant in 7 to 10 days for repeat renal panel and blood pressure check.
2. Strict low-sodium, renal-diabetic diet (sodium < 2000 mg/day).
3. Emergency Precautions: Present immediately to emergency department if experiencing chest pressure, severe shortness of breath, sudden visual changes, or systolic blood pressure > 170 mmHg.

Discharging Physician: Dr. ClinIQ Consultant, MD

Discharge Date: 2026-03-14 14:00 EST